

Your Guide to the **Medical** **Career Pathway in New** **Zealand**

**Why New Zealand? Understanding the Opportunity
for International Medical Graduates**



Chapter 1: Why New Zealand?

What You'll Learn in This Chapter

In this chapter, you'll understand why New Zealand is a strong destination for international medical graduates. We break down the country's reliance on IMGs, current workforce shortages, and where demand is highest across regions and specialties.

You'll learn how the New Zealand healthcare system is structured, the role of Te Whatu Ora as a single national employer, doctor-to-population ratios, salary benchmarks, and how IMGs are embedded in both urban and regional services.

 **Important Note:** This chapter also explains why retention challenges, training capacity limits, and future retirement trends continue to create long-term opportunities for internationally trained doctors in New Zealand.

The New Zealand Advantage for IMGs

Why New Zealand is Uniquely Positioned for IMGs

New Zealand is one of the most IMG-dependent healthcare systems in the developed world. That dependency translates directly into opportunity for you.

- Internationally trained doctors make up around **43% of New Zealand's medical workforce** compared to other countries
- In recent years, approximately **70–71% of new doctor registrations** have been international medical graduates
- New Zealand's hospital services are facing workforce shortages, particularly in **Radiology, diagnostic services, perioperative allied roles**, and key medical specialities such as General Medicine and Oncology
- These shortages are disrupting patient flow and causing delays in diagnosis, treatment, and discharge
- New Zealand is focusing international recruitment on areas with the most severe shortages and limited local supply
- Growth in doctor numbers is now largely driven by internationally trained doctors, making IMGs **central to sustaining the health system**

43% IMG Workforce

Among the highest IMG dependency rates in the developed world

70–71% New Registrations

Majority of new doctor registrations are international graduates

Critical Shortages

Radiology, General Medicine, Oncology and more

Single National Employer

Te Whatu Ora provides consistent contracts and pay nationwide

Key Distinctions from Other Global Medical Career Pathways

New Zealand offers a different value proposition compared to Australia, the UK, USA, and Canada:

Lower Total Pathway Cost

The combined fees for NZREX and associated MCNZ processes are comparatively lower once exam, portfolio, and clinical exam fees are aggregated

Multiple IMG-Specific Pathways

MCNZ formally recognises several routes for IMGs – NZREX Clinical, Competent Authority (UK/Irish/US/Canadian/Australian experience), vocational/specialist pathways, and comparable health system routes

Smaller, More Integrated System

New Zealand has one national health employer, Te Whatu Ora, with identical contracts, pay, and employment terms across the country – no state-based variations to navigate

 **Important Context:** New Zealand trains relatively few medical students domestically compared with need, which increases dependence on IMGs but also means the system is under strain. Retention of IMGs is a challenge, with a significant proportion leaving within a few years for other countries or returning home.

Healthcare Demand and **IMG** Integration in New Zealand

Official data from MCNZ and Te Whatu Ora highlight the scale of demand:

- Total practising doctors in New Zealand with current practising certificates in 2025 was **20,530** with Indian doctors being the 3rd highest contributor of IMGs making up approximately **8% of the total doctor population**
- There are approximately **374.8 doctors per 100,000 population**, up from around 333.5 per 100,000 in 2018

The Association of Salaried Medical Specialists (ASMS) and other workforce analyses further emphasise that shortages are not evenly distributed:

52% IMGs in Rural Areas

Compared to 38% in urban areas

60%+ in Some Districts

Some districts have well over 60% of their senior medical workforce overseas-trained

8 Key Specialties

With documented workforce shortages and high IMG presence



Specialties with Workforce Shortages and High IMG Presence

The following specialties show the most significant workforce gaps in New Zealand, representing the highest demand areas for IMGs:

Specialty	Estimated Shortage	Demand Level
General Practitioners (GPs)	25.5%	Critical
Resident Medical Officers (RMOs)	19.2%	Critical
General Medicine	17.5%	High
ENT	14.3%	High
Radiology (including Interventional)	13.9%	High
General Surgery	13.6%	High
Psychiatry	11.6%	Significant
Orthopaedic Surgery	6.2%	Moderate

Key Insight: GPs face the most acute shortage at 25.5%, followed by RMOs at 19.2%. These two categories alone represent the largest entry points for IMGs into the New Zealand system.

The New Zealand Healthcare Landscape

To decide whether New Zealand is the right destination, it helps to understand how the system is built and where IMGs fit within it.

From DHBs to Health New Zealand (Te Whatu Ora)

Until mid-2022, healthcare delivery was organised through 20 District Health Boards (DHBs). On 1 July 2022, the system was fundamentally restructured:

Before July 2022

20 District Health Boards (DHBs) – fragmented regional governance of hospital and specialist services

1 July 2022: Te Whatu Ora Created

DHBs replaced by Te Whatu Ora Health New Zealand – a single national organisation responsible for planning and delivering hospital and specialist services

Te Aka Whai Ora Partnership

Te Whatu Ora works alongside Te Aka Whai Ora (the Māori Health Authority) to improve equity and outcomes for Māori and to redesign services where needed

Unified National System

A unified employer for most hospital doctors, with national-level workforce planning and formal health workforce plans identifying shortages and projected demand

Public, Private, Primary Care and Specialist Services

New Zealand spends around **9% of GDP on health**, financed mainly through general taxation, with a mix of public and private expenditure.

Public Hospital Sector (Te Whatu Ora)



- Provides emergency care, inpatient services, and most complex specialist interventions
- Funded primarily through general taxation and Vote Health
- Care in public hospitals is **free at the point of delivery** for eligible residents
- Public hospital specialist salary is around **NZD 230,000 (INR 1.28 Cr) per year**

Primary Care (General Practice)

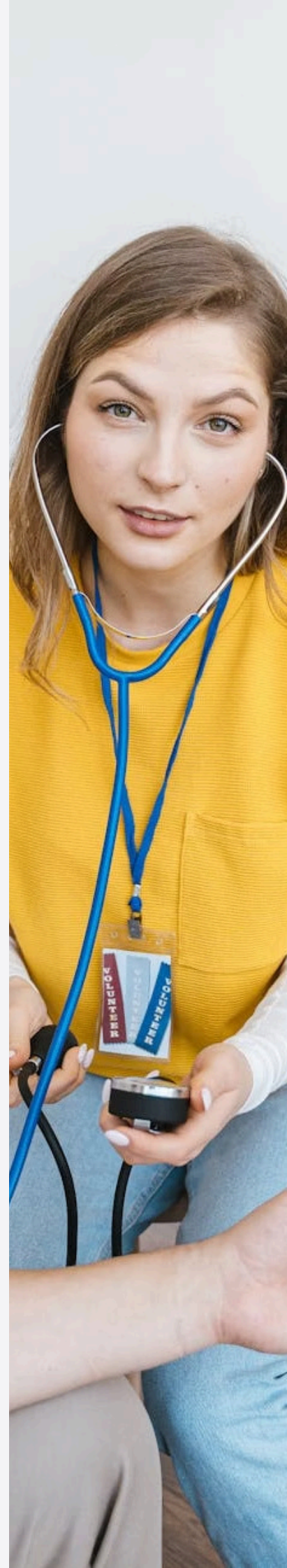


- Delivered largely by independent GP practices receiving government subsidies and patient co-payments
- GPs act as gatekeepers to most specialist and hospital services
- About **50% of a GP's income** comes from government-determined capitation payments through PHOs
- GPs have an average income of **NZD 180,000–200,000 (INR 1 Cr – 1.11 Cr) per year**; GP clinic owners earn more

Private Sector



- Plays a complementary role but does not replace the public system's core obligations
- Private specialists mostly concentrated in urban areas and set their own fees for consultation
- Hospital-based specialists (consultants and SMOs) are primarily employed by Te Whatu Ora, with some working in the private sector



Doctor-to-Population Ratios and IMG Penetration by Region

MCNZ workforce reports show that IMGs are not evenly spread across New Zealand's regions:

Region Type	IMG Representation	Examples
Urban regions	~35–40%	Auckland, Capital Coast and Hutt Valley (Wellington)
Smaller and rural regions	50–60%+	Whanganui, South Canterbury, West Coast, Tairāwhiti, Hawke's Bay

What This Means for You:

Open to Regional Locations?

You will be joining regions where IMGs are essential to keeping healthcare services running – not just filling short-term gaps. Regional hospitals are more affected when doctors leave.

Te Whatu Ora's Priority

Te Whatu Ora and medical bodies focus strongly on international recruitment and improving doctor retention, particularly for smaller and rural regions where the impact of losing one doctor is significant.

IMG Penetration by Specialty

The MCNZ workforce survey and ASMS specialist workforce projections highlight that IMGs are especially concentrated in certain specialties:

Specialty	IMG Representation (%)
Emergency Medicine	58.5%
Cardiothoracic Surgery	57.5%
Psychiatry	57.5%
Neurosurgery	55.0%
Urgent Care	52.7%
Intensive Care Medicine	52.1%
Vascular Surgery	51.5%
Occupational Medicine	51.3%

The Structural Gap

Why New Zealand Needs International Medical Graduates

Current Workforce Challenges – How Big Is the Gap?

New Zealand's dependence on IMGs is not accidental. It results from four structural and systemic forces that have been building for decades and show no signs of self-correcting.

1. Government Funding Cap

The Primary Constraint – hard annual limit on medical student training places

2. Limited Clinical Placement Capacity

Supervisor capacity constraints restrict expansion of training

3. Doctor Retention Challenge

29% of NZ graduates moved overseas in the last 10 years

4. Future Workforce Risks

Mass retirement wave approaching in GPs and rural hospital doctors

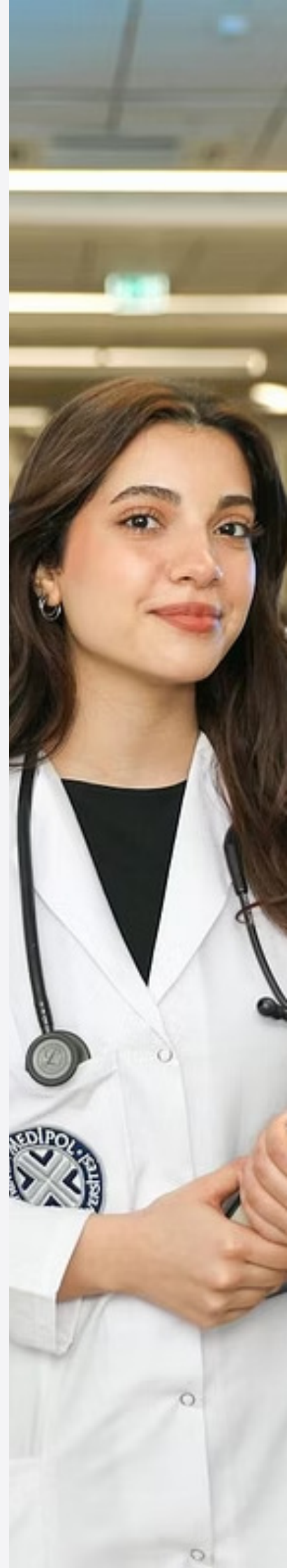
Reason 1: Government Funding Cap

The New Zealand government places a hard cap on how many medical students can be trained each year due to the high cost of medical education. Universities cannot expand without explicit government approval and funding.

Year	Training Places
2024	539 places (cap)
2025	589 places (increased)
2026 (expected)	+100 additional training places
2027 (projected)	+200 places added

Reason 2: Limited Clinical Placement Capacity

- Medical students require supervised placements in hospitals and GP practices, but **supervisor capacity is limited**
- Increasing student numbers without enough placements strains training quality
- New Zealand has a **shortage of ~1,700 doctors**, projected to reach **~3,400 by 2032**
- Service pressure leaves doctors with limited time to supervise and train students and house officers



Reason 3: Doctor Retention Challenge After Qualification

Education alone cannot address long-term doctor supply. Even when New Zealand trains doctors domestically, a significant proportion leave:

29%

Moved Overseas

New Zealand medical graduates who moved overseas in the last 10 years

~1,700

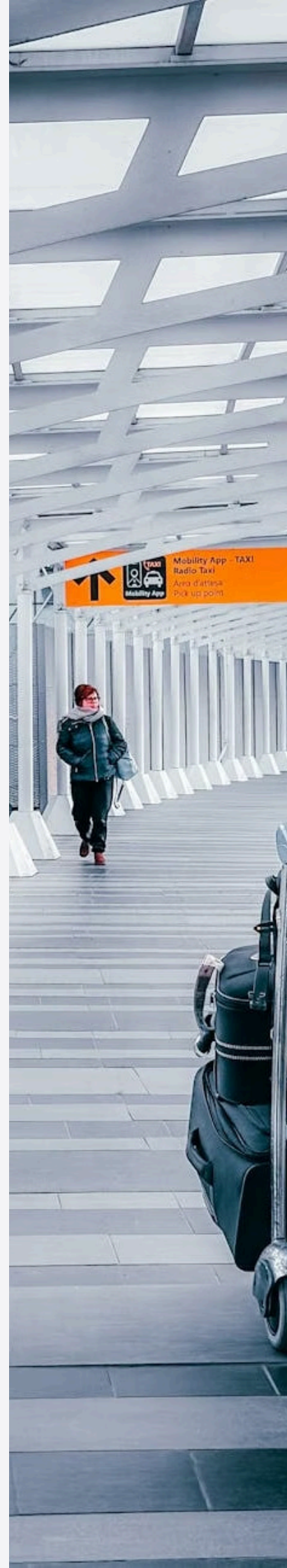
Current Shortage

Doctors currently short in New Zealand, projected to double to ~3,400 by 2032

Reason 4: Future Workforce Risks and Retirement Trends

Group	GPs	Rural Hospital Doctors
Plan to retire in 10+ years	50%	64%
Expect to retire within 5 years	35%	21%
Do not plan to leave New Zealand	69%	51%

Additional Risk: Among GPs aged 40–44, 17% are considering leaving New Zealand, with 6% planning to leave within 5 years (2024 data).



Key Takeaways from **This Chapter**



Heavy Reliance on IMGs

New Zealand's healthcare system depends heavily on IMGs, with international doctors forming a critical part of both urban and regional services. Around 43% of the workforce and 70–71% of new registrations are internationally trained.



Persistent and Structural Shortages

Workforce shortages are persistent and structural, driven by limited training capacity, uneven regional distribution, and upcoming retirement trends. The gap is expected to grow from ~1,700 to ~3,400 doctors by 2032.



Single National Employer

Te Whatu Ora operates as a single national employer, creating a unified system with consistent contracts, pay structures, and workforce planning – unlike multi-state systems in Australia or the US.



Highest Demand Areas

Demand for IMGs is highest in general medicine, psychiatry, radiology, surgery, general practice, and resident medical officer roles, especially outside major cities.



Start With the Right Pathway

Success in New Zealand starts with choosing the right registration pathway early, based on your training background, experience, timelines, and long-term goals.

Coming Up in Chapter 2:

Registration Pathways in New Zealand

Now that you understand why New Zealand is a compelling destination for internationally trained doctors, the next step is understanding how to get registered.

In the Next Chapter, You'll Understand:

- **Main MCNZ Registration Pathways**
The pathways available to IMGs through the Medical Council of New Zealand
- **NZREX, Exam-Based Routes, and Australia/UK Pathways**
How different routes compare and which suits your background
- **Key Eligibility Rules, Timelines, and Costs**
The critical factors that will affect which pathway you can and should pursue
- **Identifying Your Most Realistic Pathway**
How to match your training background and experience to the right registration route

✓ **Your Next Step:** Choosing the right MCNZ registration pathway is one of the most consequential decisions in your New Zealand medical journey. Chapter 2 gives you everything you need to make that choice with confidence.

